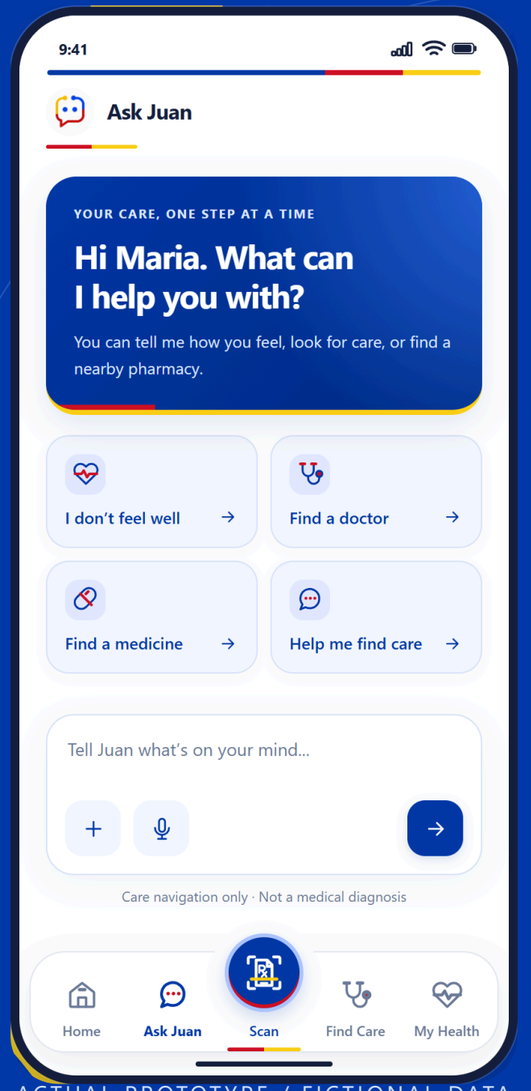
ALAGANG ABOT-KAMAY

One concern. A clearer path to care.

From finding appropriate care to locating prescribed medicine.
One proposed journey for Davao City.

> **Easier for residents.**
> **Accountable for sponsors.**



ACTUAL PROTOTYPE / FICTIONAL DATA

Getting healthcare shouldn’t mean figuring out everything alone.

01

Where do I go?

Find the starting point

02

Which provider?

Choose appropriate care

03

Am I covered?

Understand eligibility

04

Where is my medicine?

Find prescribed medicine

1,848,947

Davao City residents
2024 census

41.2%

PH current health spending paid out of pocket
2025

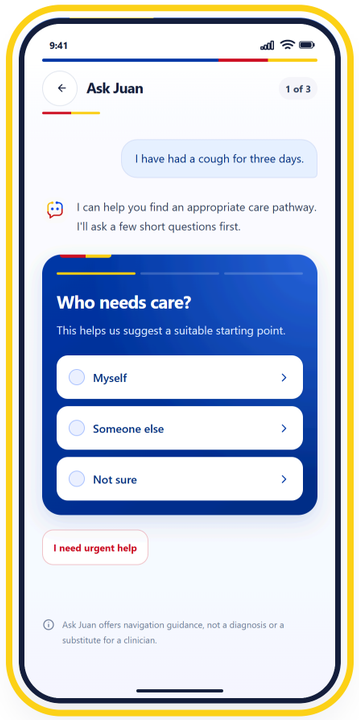
Better coordination may make existing care easier to access.

Context, not proof of demand or measured access friction. PSA 2024 census · PSA 2025 PNHA

One guided journey. From concern to care.

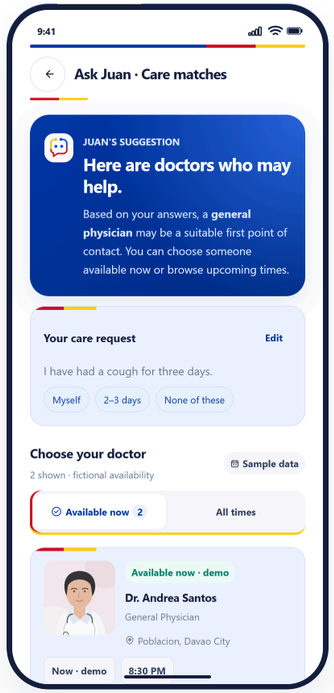
Real prototype screens. The connected service is proposed.

01 ASK



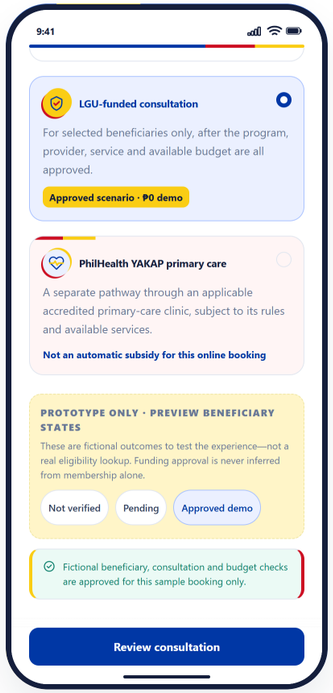
Non-emergency intake.
Guidance without diagnosis.

→ 02 MATCH



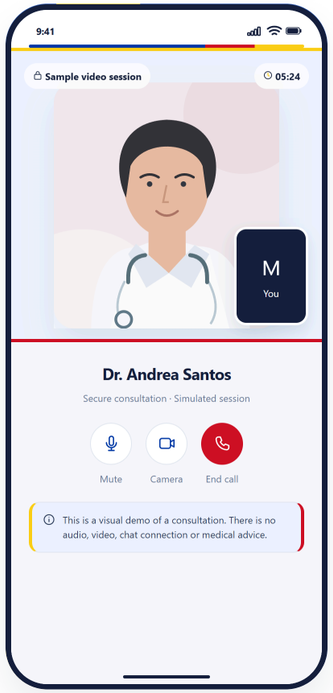
Suitable care options.
Public / covered first.*

→ 03 CHECK



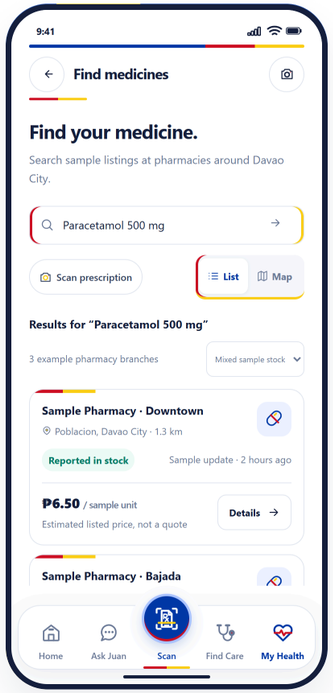
Sponsor eligibility rules.*
Visit & budget controls.*

→ 04 CONSULT



Appointment request.*
Consent-based handoff.*

→ 05 LOCATE



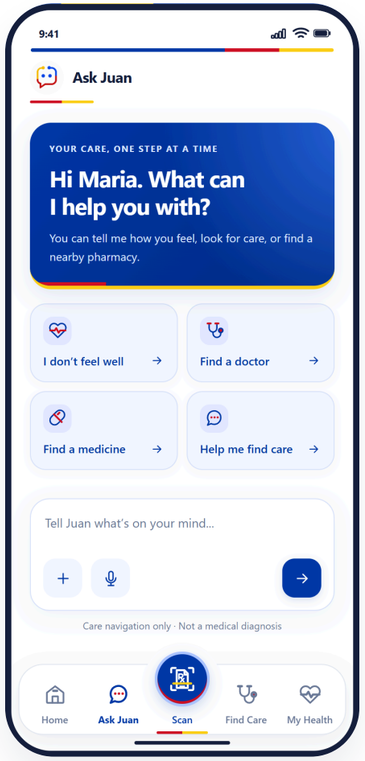
Prescribed medicine search.
Confirm pharmacy stock.*

Existing prescription? Start with medicine search.

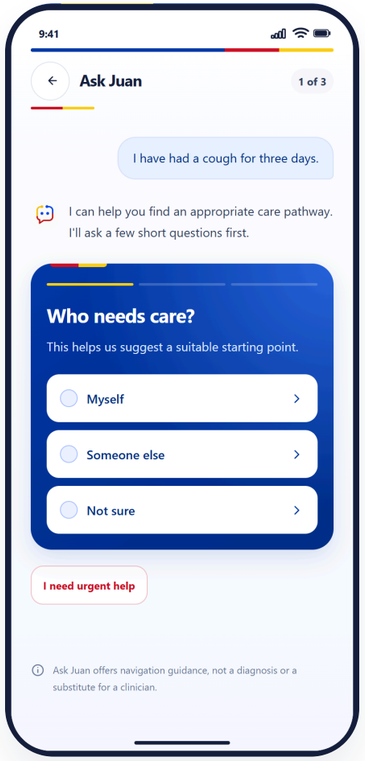
Sponsors see eligibility, booking & spending records.*

Fictional data throughout. *Proposed connected behavior. No live care, eligibility, handoff, stock or sponsor integration. Clinicians diagnose and prescribe.

Describe a concern.
Find the next step.



Choose a starting point



Answer three short questions

Describe a concern, find a doctor, or start with medicine search.

01 **Who needs care?**

Identify who the concern is for.

02 **How long has it lasted?**

A short intake organizes the concern.

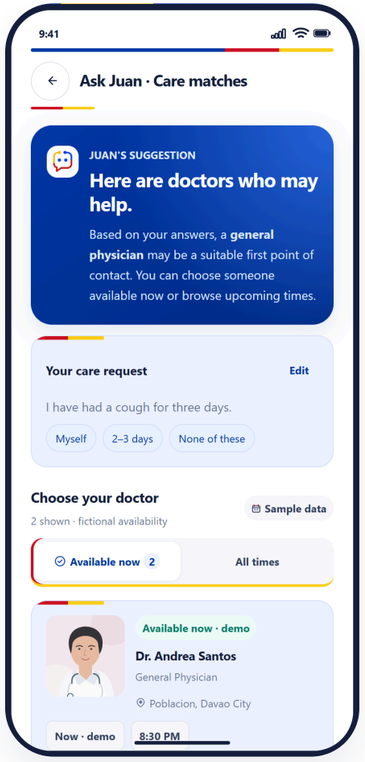
03 **Any warning signs?**

Urgent warning signs lead to separate urgent-care guidance.

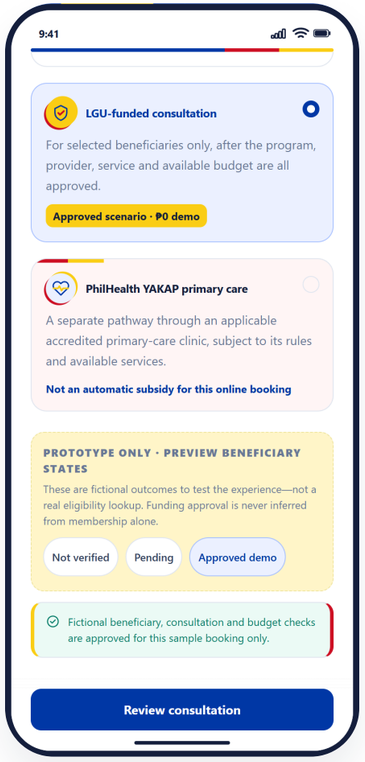
> **Juan guides navigation. A licensed clinician makes clinical decisions.**

Actual app screens and local demonstration rules. No connected AI model, diagnosis, prescribing or emergency service.

Choose care.
Understand who pays.



Compare doctors & sample slots



Review separate care pathways

The proposed connected service would prioritize available public or covered care, then relevant participating providers.

01 **Standard care**

The prototype can continue with the displayed sample fee.

02 **Sponsored care, when approved**

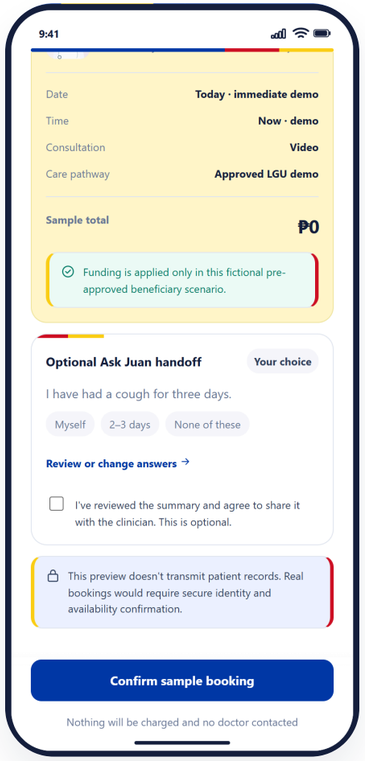
A future program must check beneficiary rules, provider, service, visit allowance and remaining budget.

03 **PhilHealth YAKAP**

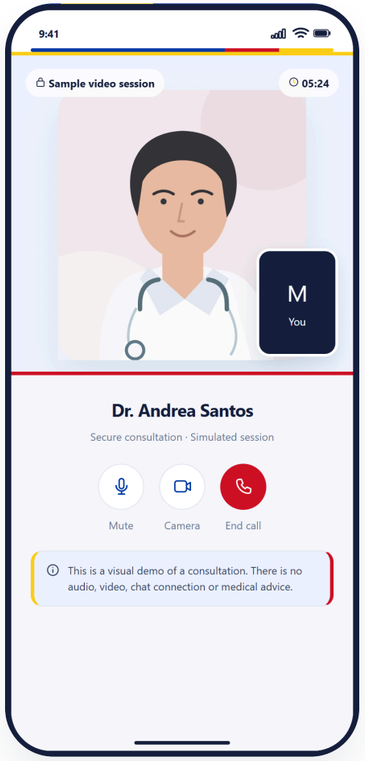
A separate primary-care pathway. Membership alone does not approve an LGU-funded consultation.

Fictional providers, availability and approval. Public-first ranking and institutional verification remain proposed.

Review the appointment. Choose what to share.



Review details & optional consent



Explore the consultation preview

01 **Confirm the care choices**

Provider, time, consultation mode, funding pathway and sample total appear before confirmation.

02 **Share the intake only by choice**

The real review screen leaves summary-sharing consent unchecked by default.

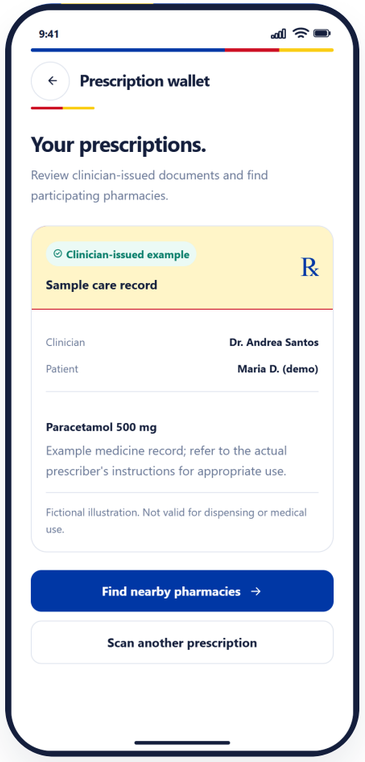
03 **A clinician retains authority**

In the proposed service, a licensed clinician reviews the concern, diagnoses and prescribes.

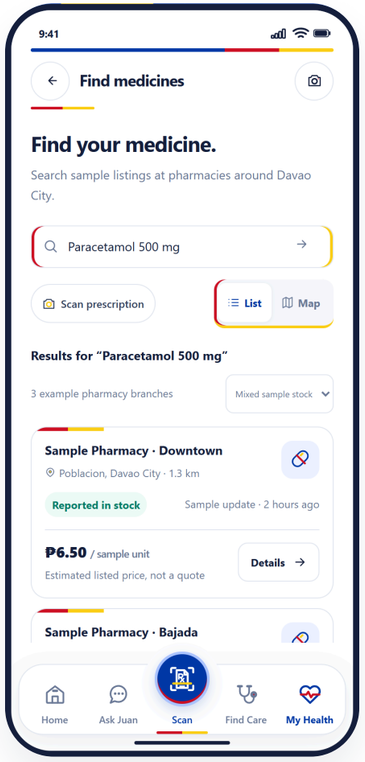
> **The current demo creates a local sample booking and simulates a consultation.**

No clinician is contacted, no patient record is transmitted and nothing is charged. Connected appointments and handoff remain proposed.

Find the prescribed medicine.
Confirm before going.



Open the example prescription



Compare nearby sample listings

Medicine search can follow a consultation or serve someone who already has a prescription.

01 **Search the medicine**

The current demo has example results for Paracetamol 500 mg.

02 **Compare the listings**

Sample branches show prices, distance, stock status and update age.

03 **Request stock confirmation**

A future pharmacy partner would confirm availability before the resident travels.

> **A reported listing is not a guarantee of current stock.**

Prescriptions, pharmacy locations, prices and stock are fictional. No inventory integration or real pharmacy confirmation is connected.

Residents need a path.
Sponsors need accountable records.

A proposed institutional license supports a program from eligibility to completed-service reconciliation.

BEFORE A SPONSORED VISIT

Approve the right use of program funding.

- Apply the institution’s beneficiary rules
- Check the approved provider and service
- Check visit allowance and budget
- Authorize eligible appointment requests

AFTER A COMPLETED VISIT

Connect the service to its spending record.

- Reconcile bookings with completed visits
- Record the approved clinical payment
- Monitor spending against program limits
- Measure staff effort and renewal value

Access follows the role.

Proposed sponsor access: eligibility, booking and expenditure records.
Clinical details stay with the patient and authorized care team, with consent-based handoff.

Proposed institutional workflow. Sponsor dashboards, audit records and production access controls are not implemented in this prototype.

An interactive prototype.
A connected service to validate.

AVAILABLE TO EXPLORE TODAY

23 screens.
One local demo.

- Guided intake and urgent-warning routes
- Doctor filters, sample slots and booking review
- Separate care pathways and optional consent
- Consultation, prescription and pharmacy previews

Self-contained HTML, CSS and JavaScript.
No account, backend or internet required.

REQUIRED FOR AN AUTHORIZED PILOT

Approved partners.
Verified operations.

- Provider and sponsor authorization
- Identity, eligibility and availability checks
- Secure clinical handoff and role-based records
- Real pharmacy confirmation and program reconciliation

Clinical, funding and data-protection arrangements must precede a connected pilot.

Current screens demonstrate the experience. Pilot evidence must establish its value.

All screenshots use fictional data. No signed partners, paying customers, deployed clinical service or demonstrated patient outcomes.

We don’t replace healthcare. We connect its moving parts.

EXISTING TELEHEALTH

NowServing mWell

Provider discovery, consultations and prescription-related services.

mWell already offers LGU community solutions.

EXISTING PUBLIC CARE

PhilHealth YAKAP Davao City CHO

Covered care, referrals and public healthcare support.

Existing services are the starting point.

COORDINATION TO VALIDATE

E-JUAN

One configurable institution-led workflow.

- Care navigation & sponsor rules
- Authorized booking & spending records
- Local medicine confirmation

Our opportunity: a better-coordinated journey.

Incremental value must beat existing platforms and in-house systems. NowServing · mWell · YAKAP · Davao CHO

Sponsors pay for coordination.
Not a cut of care.

ONE INSTITUTION / ONE ILLUSTRATIVE MONTH

CLINICAL SERVICES

₱50,000

100 consultations × ₱500

Paid separately to providers

+

E-JUAN SOFTWARE

₱10,000

Proposed monthly license

E-JUAN's illustrated revenue

=

INSTITUTIONAL SPENDING

₱60,000

Total before additional charges

Clinical care + software

₱100 / visit

Effective software cost at 100 consultations

Will the coordination justify its cost?

Eligibility controls. Better booking coordination. Reconciled service records. Spending oversight. Administrative efficiency.

All amounts hypothetical. No verified pricing, contracts or revenue. Fixed license assumed, with no percentage of professional fees.

Prove it locally.
Scale what works.

01 — 02 — 03

SIMULATE

Synthetic records.
Supervised workflow usability tests.

VALIDATE

Seek an authorized Davao City pilot.
Provider, sponsor & pharmacy partners.

REPLICATE

Expand after measurable value.
Purchasing feasibility & repeatable workflows.

---

54 Regional LGU organizations
Possible prospects. No customers.

Will qualified sponsors buy, use and renew?

- Successful healthcare navigation
- Completed consultations reconciled
- Willingness to pay and renew
- Administrative minutes per booking
- Pharmacy stock confirmation reliability

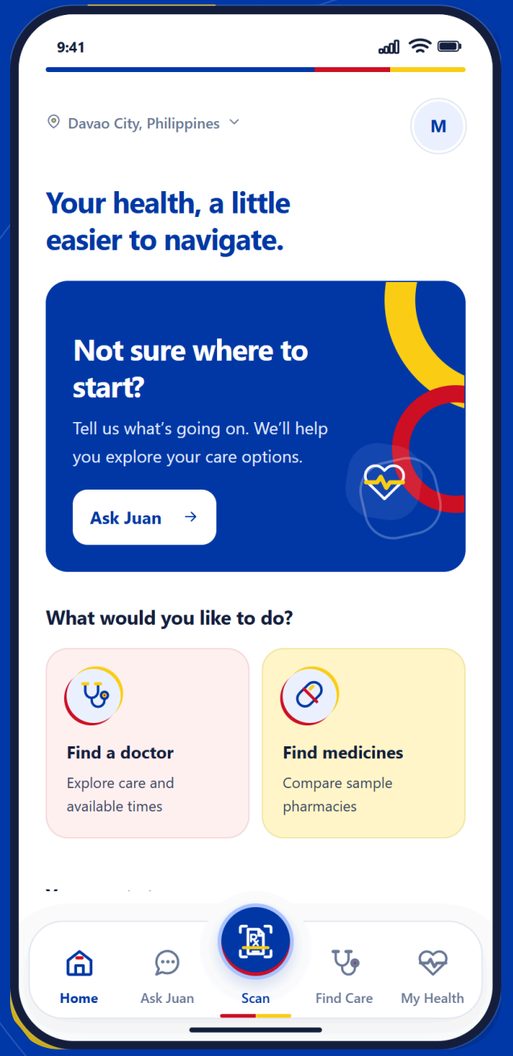
Authorized pilot subject to clinical & data-protection requirements. PSA PSGC Davao Region: 5 provinces + 6 cities + 43 municipalities.

Make every step to care count.

One coordinated journey.
More accessible navigation. Better accountability.

> **One willing healthcare provider.**
> **One authorized institutional sponsor.**
> **Pharmacy partners.**
>
> To validate one credible local pilot.

One concern. A clearer path to care.



ALAGANG ABOT-KAMAY

Concept stage. No signed partners, live clinical integrations or proven institutional demand.